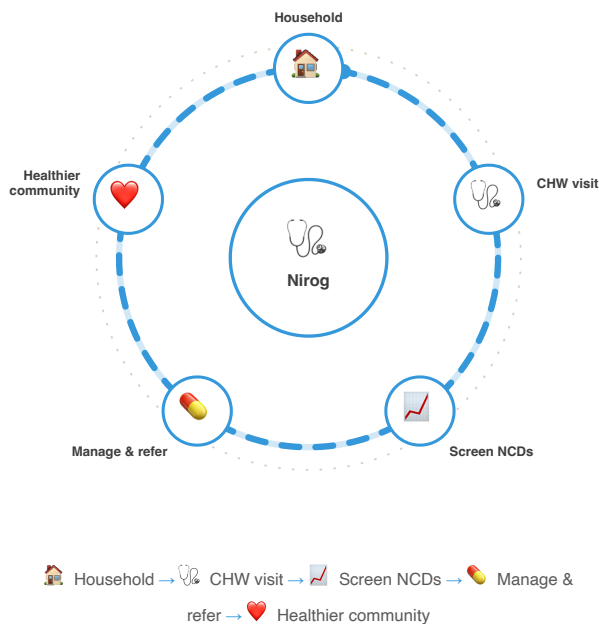


COMMUNITY HEALTH



Community health

Bangladesh's NCD crisis is huge, under-treated, and addressable by community health workers



THE SITUATION IN BANGLADESH

In Bangladesh, noncommunicable diseases now account for 67% of all deaths, and nearly one in five adults risks dying from an NCD between ages 30 and 70¹. Hypertension affects roughly a quarter of adults, yet only 36.7% are even aware they have it, just 31.1% are treated, and a mere 12.7% have it controlled². Diabetes tells the same story: around 13.1 million adults are affected, placing Bangladesh among the world's top 10 countries for diabetes, with nearly 5.7 million cases undiagnosed⁶, and 61.5% of people with diabetes unaware of their condition⁷. The gap is worst in rural areas, where undiagnosed hypertension and diabetes are widespread and regionally unequal, leaving millions to reach advanced disease before anyone detects it⁵⁹.

OUR PITCH

Nirog closes this detection-and-control gap by pairing trained community health workers with simple AI screening tools, mirroring the model proven in the COBRA-BPS cluster-randomized trial, where CHW-led multicomponent care lowered systolic blood pressure by about 5 mmHg more than usual care across rural Bangladesh, Pakistan and Sri Lanka³. That same trial was highly cost-effective, projected to cost under US\$2 per capita annually to scale, making door-to-door screening and chronic-care follow-up economically viable⁴. Our protocols follow the WHO Package of Essential NCD interventions (PEN), explicitly designed so non-physician health workers can detect and manage cardiovascular disease and diabetes in low-resource primary care⁸. The wedge is an institutional payer that funds screening and ongoing management as a cost-saving investment, because early detection and control of hypertension and diabetes is both clinically effective and cheap relative to the strokes, heart attacks and kidney failure that follow late diagnosis³⁴.

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